PRE-CLASS PREP

Sensory Perception

Know Before You Go

Read this before class — it will make the lecture click faster.

Big picture: Sensory alterations affect how patients receive and process information from the world around them. Nurses assess for deficits, manage the hospital environment to prevent deprivation and overload, and adapt communication to meet each patient's needs.

THREE TYPES OF SENSORY ALTERATION

Sensory Deficit
Reduced function in one or more senses — vision, hearing, touch, taste, smell. Can be from injury, illness, or aging. Often leads to deprivation.

Sensory Deprivation
Insufficient meaningful stimulation. Common in hospitalized patients. Causes confusion, anxiety, hallucinations, depression. Nursing: encourage family, orient to time/day, maintain day/night cycles.

Sensory Overload
Excessive uncontrollable stimulation. Causes agitation, confusion, heightened pain. Nursing: consolidate care, silence unnecessary alarms, offer earplugs and eye mask at night.

VISION & HEARING IMPAIRMENT

- **Vision types:** presbyopia (age-related near vision loss), cataracts (lens clouding), glaucoma (peripheral vision lost first), macular degeneration (central vision loss), diabetic retinopathy
- **Vision nursing:** orient to room verbally; use clock positions for food; keep call light and glasses within reach; ensure adequate lighting
- **Hearing types:** sensorineural (most common), presbycusis (age-related), conductive (trauma, wax, blockage); also tinnitus and ototoxicity
- **Hearing nursing:** face patient directly; speak clearly at normal pace — do not shout; reduce background noise; ensure hearing aids are in and working

TOUCH ALTERATIONS

- **Tactile hypersensitivity:** overly sensitive to touch — may be unable to tolerate clothing or light contact
- **Tactile hyposensitivity:** cannot feel temperature extremes or pain — may not notice injury
- **Peripheral neuropathy:** damage to peripheral nerves; common in diabetes — loss of protective sensation increases injury risk

APHASIA — LANGUAGE DISORDER, NOT COGNITIVE IMPAIRMENT

Expressive (Broca's)
Understands language but cannot produce words; halting, effortful speech; patient is aware and frustrated
Approach: yes/no questions, picture boards, patience — never finish their sentences

Receptive (Wernicke's)
Produces fluent speech that may not make sense ("word salad"); difficulty understanding spoken or written language
Approach: short simple sentences, gestures, demonstration

Global
Both expression and comprehension severely impaired; most severe; typically from large stroke
Approach: nonverbal communication, consistent caregivers, picture-based tools

- For all types: speak at normal pace — do not shout; involve speech therapy early; document what communication strategies work